1. Sympathomimetics and alkalinization of urine with or without urethral catheterization, OR
2. Induced ejaculation, OR
3. Surgical sperm retrieval.

C. Men with aspermia may be treated with:
1. Surgical sperm extraction, OR
2. Induced ejaculation (sympathomimetics, vibratory stimulation or electroejaculation).

Note: Surgical sperm aspiration is not indicated in the absence of azoospermia.

Cryopreservation (High Option, apart from Iatrogenic Infertility)

Embryo or mature oocyte cryopreservation is considered medically necessary for the following:

A. Prevention of ovarian hyperstimulation syndrome
B. Elective single embryo transfer to freeze and store supernumerary embryos or otherwise when there are supernumerary embryos
C. Pre-implantation genetic testing
D. Presence of poor endometrial development
E. When there is a failure to obtain sperm at the time of a fresh ART cycle at egg retrieval
F. Freeze only cycles:
1. All embryos are cryopreserved with the intent for subsequent transfer within a 12-month time period
G. Iatrogenic Infertility (see criteria for Iatrogenic Infertility)

Sperm cryopreservation is considered medically necessary for the following:

A. Medically necessary surgically obtained (TESE, PESE, etc.) sperm
B. Iatrogenic infertility (see criteria for Iatrogenic Infertility)

When embryo or mature oocyte cryopreservation is not covered:

A. For the purpose of embryo or oocyte accumulation or banking
B. Fresh oocyte retrievals are not indicated when previously frozen oocytes (M2) or embryo of at least BB grading quality (or equivalent) are available for transfer and if tested, are genetically normal. A fresh cycle is indicated when there are <8 previously frozen oocytes (M2) as long as those oocytes are not being used in conjunction with a fresh cycle.

Elective Single Embryo Transfer (eSET) (High Option)

A. Elective single embryo transfer (eSET) is considered medically necessary for all patients with a favorable prognosis.